What scaling up RHD screening and prophylaxis might buy in Indonesia and Uganda: preliminary results from a first-diagnosis model

Technical review of the rheumatic heart disease investment-case model

William Garcia

University of Washington · World Bank

November 2026

These results are preliminary: the first-diagnosis model passes its structural checks but fails its fit checkpoint in both countries

Checkpoint	Indonesia	Uganda
Structural validity (accounting, D-only, one-time diagnoses)	PASS	PASS
Numerical validity (starts converge; 2× burn-in)	PASS	PASS
Epidemiological fit, every scored year 2010–2019	**FAIL**	**FAIL**
Future plausibility (2026 baseline)	**FAIL**: deaths 4.4× GBD; D share 18%	**FAIL**: deaths 2.2× GBD; D share 2%
Baseline label saved with the run	INADEQUATE	INADEQUATE

Statuses read from the saved calibration and model outputs. 05–08 were run under an explicit user override of the fit gate (5 Nov 2026). Confirmatory echo is not costed. Tolerances: $|\log \text{ratio}| < 0.15$ (prevalence), 0.25 (deaths).

We ask what scaling up echo screening and prophylaxis would buy, and at what cost

1. **Health:** how many rheumatic heart disease (RHD) deaths does scale-up avert in Indonesia and Uganda, 2026–2100?
2. **Services:** how many screens, confirmatory echoes, first diagnoses and prophylaxis person-years does it add?
3. **Value:** does the monetised benefit exceed the incremental cost?

Scope: national, both sexes, ages 0–100+; secondary prevention (screening, confirmation, benzathine penicillin G (BPG) prophylaxis) only.

About 2.2 million people live with RHD in the two countries, and most are undiagnosed in the model

2.16 million

people living with RHD in Indonesia and Uganda, 2023

Modelled share of classifiable RHD with a confirmed diagnosis entering 2026: 23% in Indonesia, 18% in Uganda; on BPG 13% and 14%.

GBD 2023 RHD prevalence, published counts summed over age and sex (Indonesia 1,400,743; Uganda 762,663). Model shares are end-of-2026 reference-arm stocks, grown from provisional initial care shares. Amber marks provisional assumptions throughout.

The model builds on an established RHD investment case, but its structure and parameters are provisional

Builds on

- Investment-case design of Coates et al. (2021): in the African Union, secondary prevention with secondary and tertiary care returned a benefit–cost ratio (BCR) of 4.7 to 2030
- World Heart Federation (WHF) echo stages A–D (Rwebembera et al., 2023)
- GBD 2023 burden; UN World Population Prospects (WPP) 2024

Boundary

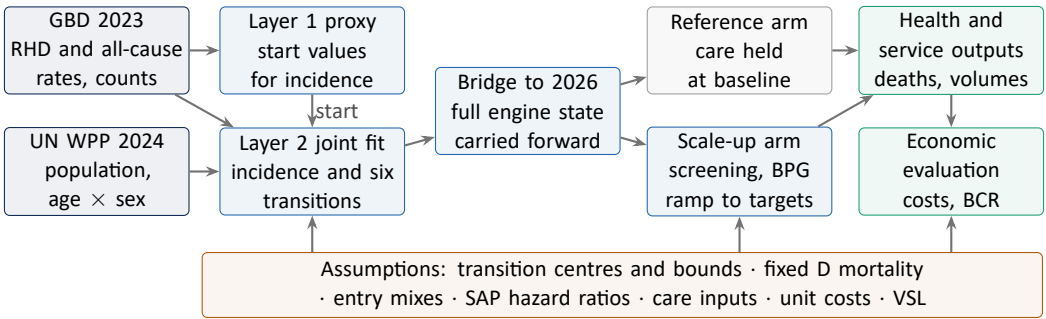
Deterministic point estimates; no sensitivity analysis.

Disease history and first confirmed diagnosis are separated; stage-D-only mortality and a progression effect of SAP are provisional choices.

The fit fails its tolerances; results are **not** a credible baseline yet.

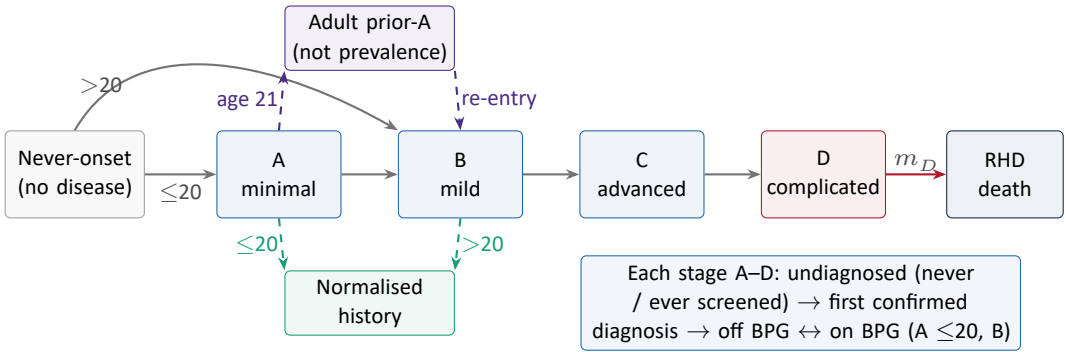
How the model works

Four linked steps turn GBD and WPP inputs into health and economic contrasts



GBD supplies the targets; WPP the population. Layer 2, the bridge and both arms run one annual engine (abcd_first_diagnosis_v1). SAP: secondary antibiotic prophylaxis. VSL: value of a statistical life.

New disease enters A before age 21 and B after; screening finds it, and only stage D dies of RHD



Regression C→B and B→A at ages ≤20; other-cause death from every state; D→C fixed at 0

Drawn from the saved specification: new disease enters A at ages ≤ 20 and B above. Ages are attained ages; the boundary is a modelling convention, not a WHF rule. Surgery is a service on C→D and D death, not a state.

Every stage is split by care status, and only some stocks count as prevalence or receive SAP

Stock	Stages	Prevalence	SAP	Screenable
Undiagnosed, never screened	A–D	Yes	No	Yes
Undiagnosed, screened (missed / unconfirmed)	A–D	Yes	No	No (one screen)
Confirmed, not on BPG	A–D	Yes	Eligible if A ($\leq$20) or B	No
Confirmed, on BPG	A ($\leq$20), B	Yes	Yes	No
Adult prior-A (age $>$20)	–	No	No	If never screened
Normalised history	–	No	No	If never screened
Never-onset	–	No	No	If never screened

Classifiable RHD (GBD prevalence mapping) = undiagnosed + diagnosed in A–D; RHD deaths come from diagnosed and undiagnosed D. A first confirmed diagnosis happens once; nobody returns to undiagnosed.

Six transitions are calibrated; B$\rightarrow$C and C$\rightarrow$D are shared by children and adults

Annual probability	Age $\leq$20	Age $>$20	Indonesia	Uganda
A $\rightarrow$ normalised	calibrated	0	0.242	0.348
A $\rightarrow$ B (adult: prior-A $\rightarrow$ B)	calibrated	same value	0.250	0.250
B $\rightarrow$ A	calibrated	0	0.053	0.159
B $\rightarrow$ C	calibrated	same value	0.036	0.004
C $\rightarrow$ B	calibrated	0	0.025	0.046
C $\rightarrow$ D	calibrated	same value	0.300	0.229
B $\rightarrow$ normalised	0	fixed	0.020	0.020
D $\rightarrow$ RHD death	fixed	fixed	0.119	0.060

Values in force (calibrated or fixed). Annual probabilities are converted once to hazards. A$\rightarrow$C, A/B/C RHD death and D$\rightarrow$C are exact zeros. The former adult progression and late-entry D-death multipliers are removed (value 1).

At age 21, surviving A becomes prior-A: about 6,800 people a year in Indonesia, 5,100 in Uganda

Reference arm, 2026	Indonesia	Uganda
New modelled disease (A $\leq$20, B $>$20)	58,737	63,793
A reclassified to prior-A at 21	6,770	5,142
Prior-A re-entering B	6,769	4,876
Adult prior-A stock (end of year)	20,293	14,601
Normalised history stock (end of year)	1,152,212	1,085,300

New disease is not diagnosis. Re-entry from prior-A is tracked separately from first entry and from first diagnosis.

Competing exits are split on the hazard scale, so every stock balances exactly

Each state faces several annual risks at once; converting them to hazards keeps every outflow non-negative.

$$p_{s\to e} = \left(1 - e^{-H_s}\right) \frac{h_e}{H_s}, \qquad h_e = -\ln\left(1 - p_e\right), \qquad H_s = \sum_{e \in \mathcal{E}_s} h_e$$

State s keeps the share e^{-H_s}. For stage B at age > 20: to C, to normalised, other-cause death (its RHD-death hazard is exactly zero). All probabilities are per year.

Each year reconciles the population, updates care, applies exits, adds new disease, then ages everyone

1. Stocks enter year t already aged; never-onset and normalised pools absorb WPP births and migration
2. Care update: screening, confirmation, clinical detection, BPG retention and initiation
3. Competing exits split on the hazard scale; on-BPG people get the SAP hazard multipliers
4. New modelled disease from the never-onset pool joins A (≤ 20) or B (> 20) at year end
5. Survivors age one year; surviving A at 20 becomes prior-A; 100+ is open

Disease and prior-A stocks are never rescaled. New disease first faces stage hazards the next year. Prevalence is observed as the mean of the start and end stocks.

A first confirmed diagnosis needs a screen, a true positive and a completed confirmation — or clinical detection

$$\mathrm{dx}_t^{\mathrm{scr}} = q_t\, s\, \kappa\, \phi\, U_t^{\mathrm{ns}}, \qquad q_t = 1 - (1 - c_t)^{1/35}, \qquad \mathrm{dx}_t^{\mathrm{clin}} = k\,(U_t - \mathrm{dx}_t^{\mathrm{scr}})$$

U^{ns}: undiagnosed, never screened; c_t: probability of one screen across ages 5–39; sensitivity s = 0.90, confirmation completion κ = 0.90, confirmation sensitivity ϕ = 1.00; clinical detection k = 0.02. Screening encounters $= q_t \times$ never-screened people without a prior diagnosis.

Perfect specificity assumed (no false positives). Window target c: reference 5% (Indonesia), 1% (Uganda); scale-up 80% by 2050 (q = 0.045 a year).

SAP lowers progression only for eligible people on BPG; adult B carries cost but no modelled benefit

	Reference	Scale-up (2050)
Eligibility	A ($\leq$20), B	same
BPG initiation (diagnosed, off BPG)	50%	90%
BPG retention (annual)	90%	95%
Adherence: good / partial share	60% / 20%	85% / 10%
Protection factor a (good + 0.5 $\times$ partial)	0.70	0.90
HR A$\rightarrow$B, age $\leq$20	0.039	same
HR B$\rightarrow$C, age $\leq$20 / $>$20	0.200 / 1	same
HR C$\rightarrow$D, D death	1 (not eligible)	same

On BPG: $h = h_0\,[1 - a\,(1 - \text{HR})]$. Child HRs from GOAL-trial arm risks (0.075 vs 0.003, 0.119 vs 0.025; provisional stage mapping), not annualised again. Both countries use the same values.

Observed data anchor demography and burden; stage dynamics and care are calibrated or assumed

Input	Source	Status
Population	UN WPP 2024	Observed
Prevalence, deaths	GBD rate $\times$ WPP	Observed
Background deaths	GBD all-cause $-$ RHD	Observed
RHD incidence	GBD $\times$ Layer-2 α	Calibrated
Six transitions	Layer 2 within bounds	Partly identified
Stage-D mortality	0.119 / 0.0597	Fixed, provisional
Entry mixes, adult normalisation	Modelling assumption	Fixed, new
Initial diagnosed / on-BPG share	5% / 4% (Indonesia)	Provisional
Incidence trend	Assumption	Provisional

Costs price screening encounters and BPG person-years; confirmation is not priced

Input	Indonesia	Uganda	Status
Screening encounter	US$1.10	US$0.40	Undocumented
Confirmatory echo	not costed	not costed	Omitted
BPG (SAP) person-year	US$110	US$40	Undocumented
Valve surgery	US$9,000	US$5,000	Undocumented
GNI per capita (VSL anchor)	US$4,020	US$810	World Bank, 2019
VSL, discounting	30× GNI; 3%	30× GNI; 3%	Provisional

Volumes come from the engine: encounters, on-BPG person-years (A ≤ 20 and B), surgeries. Unit-cost sources and price year are not recorded in the pipeline. Total cost is understated by the confirmation cost.

How the model was calibrated

Layer 2 fits prevalence 2.5 times as heavily as deaths, with child and adult components weighted equally

Layer 1: aggregate proxy (2000–2019)

- Well–sick–dead proxy; GBD counts
- Weights incidence / prevalence / deaths 1 / 2.5 / 1
- Role: start values only

Layer 2: production engine (scored 2010–2019)

- Prevalence 2.5, deaths 1.0, incidence 0.0
- Each split 0–19 / 20+ at 0.5 / 0.5
- Free: 6 transitions + 12 incidence α
- Burn-in 120 cycles, per-capita balanced

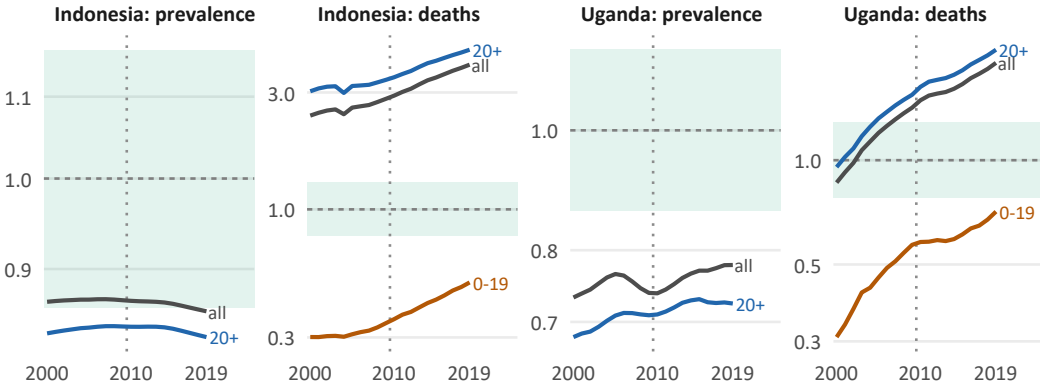
Targets: GBD rate $\times$ WPP population; prevalence = classifiable RHD (diagnosed + undiagnosed), prior-A and normalised excluded. Prior weight $\lambda = 0.05$.

Deaths are 2.91–3.90 times GBD in Indonesia and 1.49–1.91 in Uganda across 2010–2019; no checkpoint quantity is in tolerance

	Indonesia		Uganda	
Model / GBD target	Ratio range	Yrs in tol.	Ratio range	Yrs in tol.
Prevalence, all ages	0.86–0.87	7/10	0.74–0.78	0/10
Prevalence, 20+	0.83–0.84	0/10	0.71–0.73	0/10
RHD deaths, all ages	2.91–3.90	0/10	1.49–1.91	0/10
RHD deaths, 0–19	0.36–0.50	0/10	0.58–0.71	0/10
RHD deaths, 20+	3.46–4.48	0/10	1.63–2.08	0/10
Prevalence, 0–19 (diagnostic)	1.07–1.08	10/10	0.80–0.92	4/10
Incidence (validation)	1.02–1.02	–	0.64–0.65	–

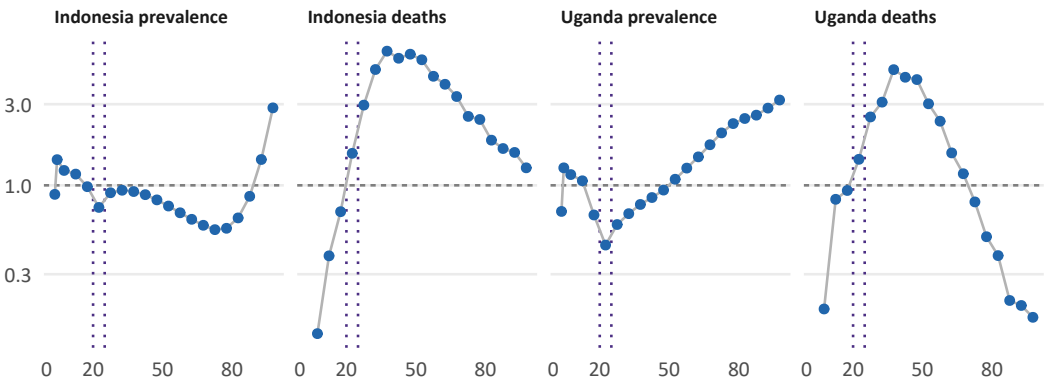
Ratios over the scored years, both sexes, GBD rate $\times$ WPP population. Tolerance $|\log \text{ratio}| < 0.15$ (prevalence) and 0.25 (deaths) in every year. Ratios to published GBD counts are in backup.

Over 2010–2019 the death ratios rise while GBD deaths per prevalent case fall



Model / GBD target by year, log scale. Green band: tolerance; dotted line: start of the scored years. GBD deaths per prevalent case change -21% (Indonesia) and -13% (Uganda) from 2010 to 2019.

By age, prevalence dips at 20–24, where prior-A leaves the count, and adult deaths are above GBD



Model / GBD target by GBD age group, 2019, both sexes, log scale. Purple dotted lines: ages 20–24. Prevalence at 20–24 is 0.74 (Indonesia) and 0.45 (Uganda) times GBD.

Calibrated transitions hit their bounds: A → B, C → D (Indonesia); A → B (Uganda)

Annual probability	Centre [bounds]	Indonesia	Uganda
A → normalised	0.311 [0.005, 0.60]	0.2423	0.3481
A → B	0.054 [0.001, 0.25]	**0.2500**	**0.2500**
B → A	0.177 [0.002, 0.40]	0.0535	0.1591
B → C	0.074 [0.001, 0.20]	0.0358	0.0038
C → B	0.050 [0.001, 0.15]	0.0251	0.0459
C → D	0.112 [0.001, 0.30]	**0.3000**	0.2286

Amber bold: at a bound. Incidence multipliers: 0.81–1.36 (Indonesia), 0.46–0.78 (Uganda). Fixed: D death, adult B normalisation, entry mixes, D→C = 0.

Stage D holds 17% of Indonesia's modelled RHD against a GBD heart-failure proxy of 3.2%

	Indonesia	Uganda
Stage-D annual RHD death probability (fixed input)	11.9%	6.0%
Realised D death risk, 2019	11.8%	5.9%
Stage-D share of classifiable RHD, 2019	16.7%	2.1%
GBD heart-failure proxy share (diagnostic)	3.2%	0.9%
Model / GBD RHD deaths per prevalent case, 2019	4.5	2.5

D mortality is the same for every disease and diagnosis history (the former late-entry multiplier of 2 is removed). Realised risk is lower because of competing other-cause death and reference surgery.

Four mechanisms limit the fit under the fixed assumptions

Mechanism	Evidence	Why it binds
Shared progression vs childhood deaths	GBD child / adult case fatality 0.63% / 0.41% (Indonesia)	Childhood deaths need fast progression; adults then over-progress
Prior-A exclusion at age 21	Prevalence 20–24: 0.74 / 0.45 × GBD	Child A counts as RHD; the same lesion stops counting at 21
Too few adult exits	Uganda prevalence 60+: 1.72 × GBD	Adult B leaves only via 2% normalisation or progression
Falling GBD case fatality	Deaths per case -21% / -13%, 2010–2019	Constant natural history cannot reproduce a declining trend

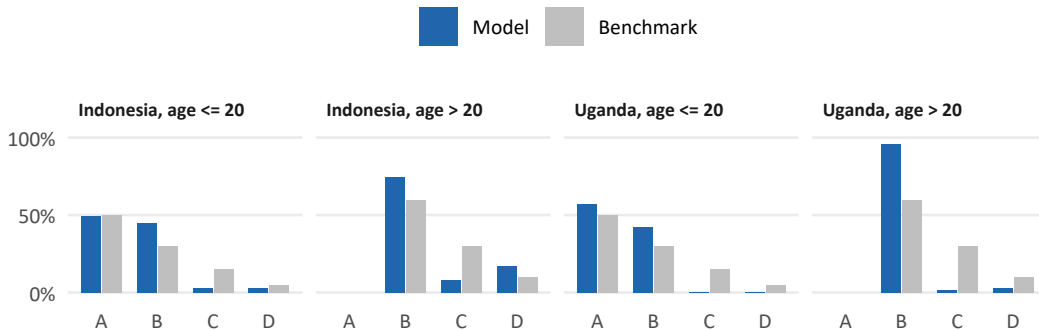
Evidence from the saved Layer-2 targets and fit (Indonesia / Uganda where two values are shown).

No tested variant passes; adult parameters plus prior-A in prevalence come closest

Variant	IDN prevalence (all / 20+)	IDN deaths (all / 0–19 / 20+)	UGA prevalence (all / 20+)	UGA deaths (all / 0–19 / 20+)	Pass?
Current spec (repo run)	0.86–0.87 / 0.83–0.84	2.9–3.9 / 0.36–0.50 / 3.5–4.5	0.74–0.78 / 0.71–0.73	1.5–1.9 / 0.58–0.71 / 1.6–2.1	no
+ calibrated adult B to C, C to D	0.75–0.76 / 0.75	1.7–2.2 / 0.89–1.26 / 1.8–2.4	—	—	no
+ calibrated adult B to normalised	—	—	0.87–0.91 / 0.83–0.85	1.5–1.9 / 0.35–0.43 / 1.6–2.1	no
+ both (3 adult parameters)	0.86–0.87 / 0.85–0.86	1.14–1.48 / 0.89–1.26 / 1.19–1.52	0.82–0.86 / 0.76–0.78	1.04–1.33 / 0.84–1.03 / 1.07–1.37	no
+ both, prior-A counted in prevalence	0.87–0.89 / 0.87–0.88 (in tol.)	1.12–1.46 / 0.86–1.22 / 1.18–1.51	0.88–0.92 / 0.86–0.88 (in tol.) (20+ marginally out in 2010: 0.860)	1.03–1.31 / 0.84–1.03 / 1.06–1.35	no

Scratch tests (one local optimisation each; not saved outputs); source: the dev/ checkpoint note.

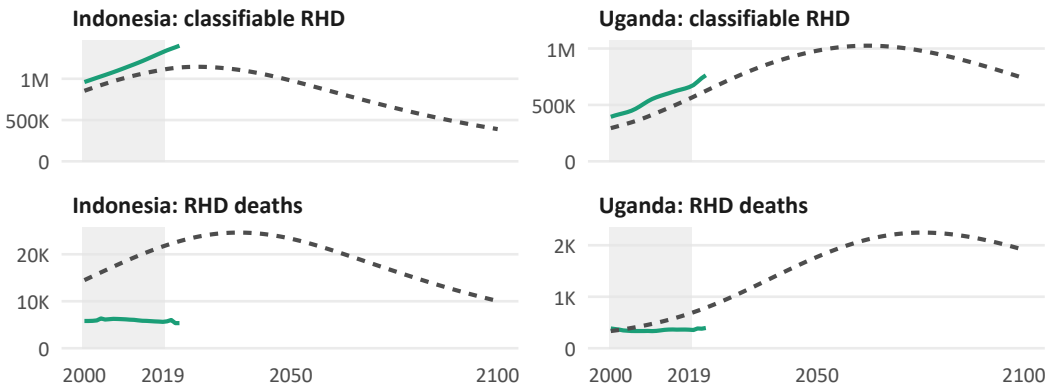
In 2010–2019, 75% of Indonesia's adult first diagnoses are stage B, against a provisional benchmark of 60%



Stage at first confirmed diagnosis (screen + clinical), reference care. Benchmark: provisional illustrative mixes, a diagnostic comparison only — not a fit term or a national stage target.

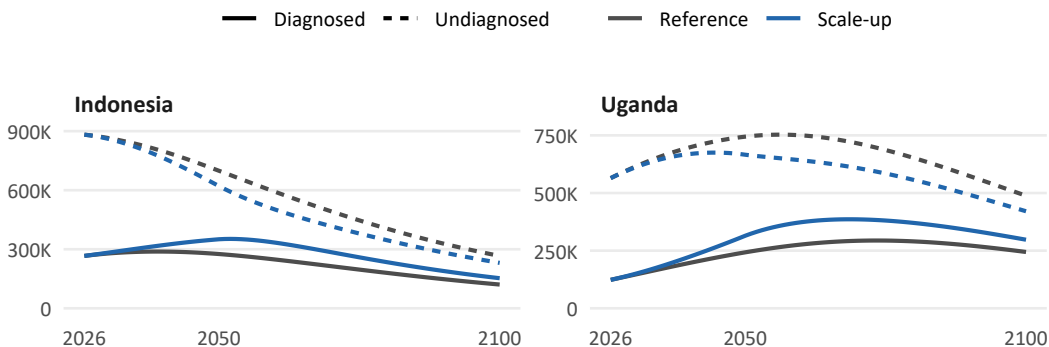
What the preliminary model projects

Projected 2026 RHD deaths are 4.4× GBD 2023 in Indonesia and 2.2× in Uganda



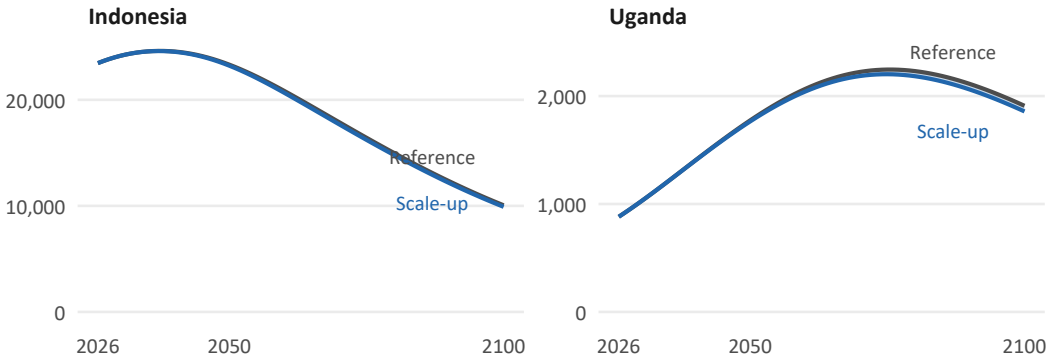
Green: GBD 2023 published counts. Dashed grey: model (calibration window 2000–2019, shaded; bridge; reference arm 2026–2100). All ages, both sexes.

Scale-up raises the diagnosed share of classifiable RHD to 36% in Indonesia and 32% in Uganda by 2050



Classifiable RHD at the end of each year: diagnosed (solid) and undiagnosed (dashed); grey reference, blue scale-up. Total classifiable RHD is nearly identical across arms.

Scale-up averts about 9,500 RHD deaths in Indonesia and 2,000 in Uganda by 2100, up to 1.5% of reference-arm deaths



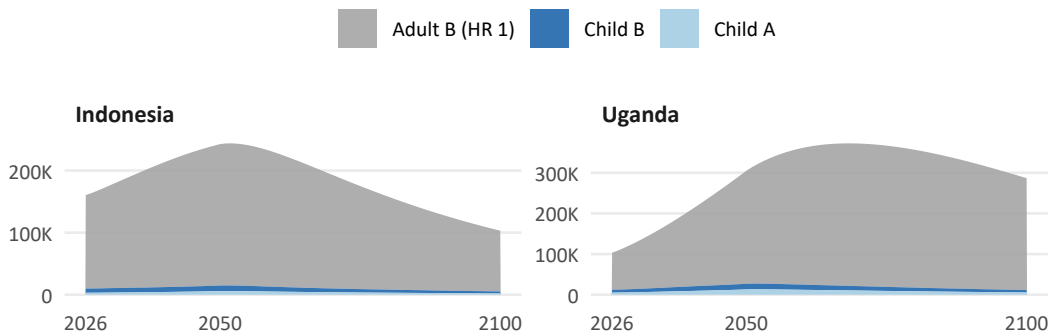
Annual RHD deaths. Cumulative reduction: 0.67% (Indonesia) and 1.46% (Uganda) of reference-arm deaths, 2026–2100; 800 and 66 of the totals occur by 2050.

By 2050 scale-up delivers 4.9 million screens a year in Indonesia but adds only 9,805 screen-detected first diagnoses

	Indonesia		Uganda	
Per year, 2050	Reference	Scale-up	Reference	Scale-up
Screening encounters	0.21 m	**4.91 m**	0.02 m	**1.80 m**
True screen positives	528	**11,422**	114	**12,912**
Confirmations completed (uncosted)	475	**10,280**	103	**11,621**
First diagnoses via screening	475	**10,280**	103	**11,621**
First diagnoses, clinical	14,091	**12,434**	14,732	**13,060**
Diagnosed classifiable RHD (end)	275,676	**350,811**	242,838	**314,927**
People on BPG (person-years)	162,686	**242,647**	204,730	**305,402**
Valve surgeries	2,106	**2,096**	401	**398**

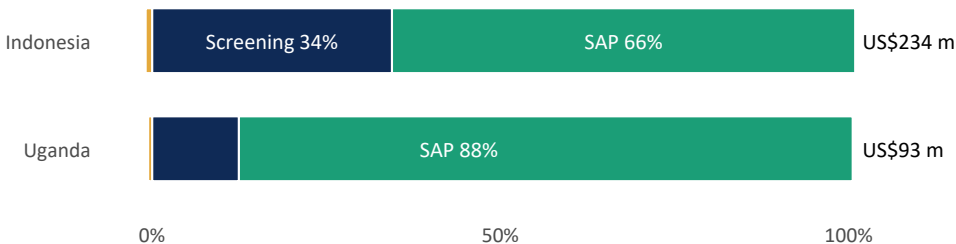
Counts from the engine's care trace; m = million. Encounters include people without RHD. On-BPG person-years: confirmed A (≤ 20) and B.

94% (Indonesia) and 91% (Uganda) of scale-up BPG person-years in 2050 are adults in stage B, with hazard ratio 1



Scale-up arm, people on BPG by stage and age (person-years). Child = age ≤ 20. Adult B on BPG is costed but carries no modelled progression benefit.

BPG person-years are 66% of Indonesia's and 88% of Uganda's discounted incremental cost



Discounted (3%, base 2026) incremental cost, 2026–2100, by component; screening 34% (Indonesia) and 12% (Uganda). Confirmatory echoes (320,353 Indonesia, 421,028 Uganda) are not priced.

Benefit–cost ratios are 6.06 in Indonesia and 0.73 in Uganda over 2026–2100 — on an inadequate baseline

Discounted to 2026 at 3%, US$	Indonesia	Uganda
Incremental cost	233.7 m	92.9 m
Monetised benefit	1,415.6 m	68.1 m
Net benefit	1,181.9 m	-24.8 m
Benefit–cost ratio	**6.06**	**0.73**
RHD deaths averted (undiscounted)	9,544	2,028
Cost per death / per DALY averted	US$24,484 / US$2,962	US$45,823 / US$6,678
Confirmatory echoes not costed (incremental)	320,353	421,028

Benefit = deaths averted × VSL only (30× GNI per capita, 2019, grown with GDP per capita). m = million. Deterministic point estimates; costs understated by the confirmation cost.

Ending the horizon in 2050 lowers the ratios to 1.16 and 0.07

	Indonesia		Uganda	
	2026–2050	2026–2100	2026–2050	2026–2100
RHD deaths averted	757	9,500	66	2,000
Benefit–cost ratio	**1.16**	**6.06**	**0.07**	**0.73**
Net benefit (discounted, US$)	15.4 m	1,181.9 m	-26.0 m	-24.8 m

2026–2050 columns are deck-only sums of the saved runs to 2050; the model runs forward only, so this equals a 2050 run with the same inputs.

What must be resolved

The engine and pipeline run end to end; the calibration and key assumptions are not settled

Implemented

- First-diagnosis engine
- Screening, confirmation, BPG cascade
- Joint calibration; bridge
- Outputs, costing, BCR

Not settled

- Fit to GBD (fails)
- Shared adult progression
- Prior-A prevalence mapping
- Adult B eligibility, effect
- Confirmation unit cost

Planned

- Recalibration decision
- Sensitivity analyses
- Report rewrite
- Adherence evidence

A credible fit needs a decision on a fixed assumption; three options were tested or considered

Option	Change	Scratch evidence
1 (recommended in the checkpoint)	Free adult B→C, C→D, B→normalised; count adult prior-A as prevalence	Prevalence within tolerance; deaths 1.0–1.5×, drifting up
2	Free the three adult parameters only; keep prior-A excluded	Deaths 1.0–1.5×; prevalence 0.76–0.87×
3	Keep the current specification; accept the fit failure	As shown in this deck

From the checkpoint note (dev/checkpoint_first_diagnosis_2026-11-05.md). Passing the death tolerance in every scored year would additionally need a historical case-fatality or care trend, or a different scoring rule.

Remaining limitations bias results in directions we can name

Limitation	Likely effect on results	What resolves it
Baseline deaths	Benefit per death unclear	Recalibration decision
Adult SAP effect	Cost without benefit	Adult evidence, eligibility
Confirmation cost	Cost understated	Unit cost for confirmatory echo
Initial care shares	Shape diagnosed share	Programme data
Transferability	Direction unknown	Country cohort or registry data
Uncertainty	Spread unknown	Probabilistic, one-way analyses

On preliminary runs, scale-up returns US$6.1 (Indonesia) and US$0.7 (Uganda) per dollar through 2100 — but the calibration fails and adult prophylaxis buys no modelled benefit, so these numbers are not yet usable.

Five collaborator decisions will define the next model version

1. Should adults progress at their own calibrated rates, and should adult normalisation be estimated?
2. Should adult prior-A count as RHD prevalence, to match the GBD case definition across age 21?
3. Should the calibration reproduce GBD's falling case fatality, or score fewer years?
4. Should adults with stage B receive BPG, and with what effect?
5. What does a confirmatory echo cost, and what completion rate is realistic?

Backup slides hold detail for questions and sit outside the 40-minute budget

- Calibration objective, cells, target basis, fit by sex
- Incidence multipliers, initialisation, convergence
- Identifiability; literature transitions; stage-D proxies
- Care trajectories by arm
- Economic parameters, detail, budget impact
- Abstract; sensitivity; discrepancies; provenance

Layer 2 minimises normalised log errors with child and adult components plus weak priors

$$L_2 = w_P\left[p\,\bar{\ell}_{P,\text{0-19}} + (1-p)\,\bar{\ell}_{P,20+}\right] + w_D\left[s\,\bar{\ell}_{D,\text{0-19}} + (1-s)\,\bar{\ell}_{D,20+}\right] + w_I\bar{\ell}_I + \lambda\Big[\sum_k \frac{(\text{logit}\,p_k - \text{logit}\,p_k^0)^2}{\sigma_p^2} + \sum_{b,s}\frac{(\ln\alpha_{bs})^2}{\sigma_\alpha^2}\Big] + \lambda_\Delta\sum_s\sum_b(\Delta^2\ln\alpha_{bs})^2$$

$\bar{\ell}$: weighted mean of $[\ln(X^{\text{mod}}+1) - \ln(X^{\text{tgt}}+1)]^2$ over unmasked cells, both sexes pooled. w_P, w_D, w_I = 2.5, 1.0, 0.0; p = 0.5, s = 0.5; λ = 0.05, λ_Δ = 0.5; σ_p = 2, σ_α = 1. Optimum L = 2.833 (Indonesia), 1.813 (Uganda); prior / data 0.05 and 0.30.

Childhood deaths are the largest weighted term in Indonesia's objective

Term at the optimum	Indonesia	Uganda
Prevalence 0–19 (raw)	0.042	0.094
Prevalence 20+ (raw)	0.153	0.466
Deaths 0–19 (raw)	3.522	0.555
Deaths 20+ (raw)	1.373	0.843
Prevalence (weighted)	0.243	0.701
Deaths (weighted)	2.448	0.699
Prior: transitions	0.095	0.164
Prior: incidence α	0.020	0.169
Smoothness: α	0.027	0.081

Cells: prevalence 100 + 320, deaths 100 + 315 (Indonesia); deaths 80 + 310 (Uganda).

Layer-2 targets use GBD rates times WPP population, which differ from published counts by up to 10%

	Indonesia		Uganda	
Measure	Range	Window	Range	Window
Deaths	0.958–1.020	0.973	0.977–1.084	1.031
Incidence	0.964–1.033	0.997	0.933–1.013	0.981
Prevalence	0.966–1.025	0.983	0.904–1.014	0.974

Ratio of GBD published Number to GBD Rate × WPP population, by sex and year, 2000–2019. Model / GBD Number ratios for the checkpoint quantities are on the next slide.

Against published GBD counts the fit is similar; male deaths fit worse than female deaths in both countries

	Indonesia		Uganda	
2019: target / Number	Female	Male	Female	Male
Prevalence All ages	0.87 / 0.85	0.84 / 0.82	0.83 / 0.87	0.73 / 0.81
Prevalence 20+	0.84 / 0.83	0.82 / 0.80	0.77 / 0.82	0.68 / 0.77
Deaths All ages	3.34 / 3.32	4.94 / 4.84	1.59 / 1.56	2.46 / 2.39
Deaths 0–19	0.43 / 0.43	0.60 / 0.60	0.64 / 0.69	0.79 / 0.83
Deaths 20+	3.77 / 3.75	5.90 / 5.77	1.71 / 1.66	2.77 / 2.66

Model / (GBD rate × WPP) and model / GBD published Number. Uganda's WPP population exceeds GBD's, so its rate-based targets are higher than published counts.

Layer-2 incidence multipliers move away from the Layer-1 start values

	Indonesia		Uganda	
Age band	Layer-1 start	Layer 2 (used)	Layer-1 start	Layer 2 (used)
0-14	0.98 / 0.97	1.36 / 1.23	0.91 / 0.91	0.78 / 0.73
15-24	0.98 / 0.97	1.04 / 0.96	0.94 / 0.93	0.59 / 0.54
25-44	0.98 / 0.98	0.88 / 0.83	0.95 / 0.95	0.51 / 0.46
45-64	0.99 / 0.99	0.84 / 0.81	0.98 / 0.98	0.54 / 0.49
65-79	1.00 / 1.00	0.83 / 0.82	1.00 / 0.99	0.64 / 0.60
80+	1.00 / 1.00	0.85 / 0.85	1.00 / 1.01	0.77 / 0.74

Female / male multipliers on the GBD incidence pattern (per total population). Incidence is validation in Layer 2: model / GBD 1.02 (Indonesia), 0.64 (Uganda) in 2019.

The burn-in converges and the bridge carries the full state to 2026 without reseeding

	Indonesia	Uganda
Burn-in cycles / doubled check	120 / 240	120 / 240
Largest change, doubled burn-in	7.3e-12	1.1e-11
Bridge prevalence / GBD, 2023	0.85	0.74
Bridge RHD deaths / GBD, 2023	4.38	2.00
Seed entering 2026 / GBD-rate prevalence	0.83	0.71
Calibration → bridge hand-off gap	0.0e+00	0.0e+00

Zero-start burn-in under 2000 inputs with per-capita balancing; bridge 2020–2025 under reference care. The seed keeps diagnosis, BPG, prior-A, normalised and screening histories.

All optimiser starts converge; the starts reach 2 (Indonesia) and 1 (Uganda) distinct optima

Country	Start	Objective	Code	Evaluations
Indonesia	prior center	2.8331	0	53
Indonesia	layer1 alpha	2.8331	0	53
Indonesia	prevalence first	3.0360	0	67
Indonesia	random 1	3.0360	0	54
Indonesia	random 2	2.8331	0	59
Uganda	prior center	1.8134	0	59
Uganda	layer1 alpha	1.8134	0	67
Uganda	prevalence first	1.8135	0	70
Uganda	random 1	1.8134	0	78
Uganda	random 2	1.8134	0	75

Bounded L-BFGS-B from the prior centre, Layer-1 values, a prevalence-first start and 2 random starts. Convergence code 0 is not fit adequacy.

A one-at-a-time sweep flags 2 (Indonesia) and 1 (Uganda) parameters as prior-dominated

Parameter	Indonesia	Uganda
A $\rightarrow$ normalised	0.27	0.18
A $\rightarrow$ B	0.09	0.09
B $\rightarrow$ A	**0.64**	0.36
B $\rightarrow$ C	0.18	0.18
C $\rightarrow$ B	**0.91**	**0.73**
C $\rightarrow$ D	0.09	0.27
Incidence α, 0-14 (F / M)	0.18 / 0.18	0.18 / 0.09
Incidence α, 15-24 (F / M)	0.09 / 0.09	0.09 / 0.09
Incidence α, 25-44 (F / M)	0.18 / 0.18	0.09 / 0.09
Incidence α, 45-64 (F / M)	0.18 / 0.09	0.09 / 0.09
Incidence α, 65-79 (F / M)	0.18 / 0.18	0.09 / 0.09
Incidence α, 80+ (F / M)	0.36 / 0.36	0.27 / 0.27

Flatness: share of a parameter's range within the near-optimal band, others at the optimum; amber = prior-dominated. Weaker than joint identification.

Care trajectories differ only in screening and BPG; surgery is identical in both arms

	Indonesia		Uganda	
Care input	Reference	Scale-up	Reference	Scale-up
One screen across ages 5–39	5%	5% → 80%	1%	1% → 80%
Clinical detection (annual)	2%	2% → 2%	2%	2% → 2%
BPG initiation (annual)	50%	50% → 90%	50%	50% → 90%
BPG retention (annual)	90%	90% → 95%	90%	90% → 95%
Good adherence share	60%	60% → 85%	60%	60% → 85%
Partial adherence share	20%	20% → 10%	20%	20% → 10%
Surgery coverage	5%	5% → 5%	3%	3% → 3%

Scale-up ramps linearly 2026–2050, then holds. The reference arm equals the care applied in calibration (checked at render).

Published annual transition probabilities vary 7- to 108-fold across studies

Transition	Studies	Minimum	Maximum	Indonesia	Uganda
A → normalised	11	0.0074	0.2490	0.2423	0.3481
A → B	15	0.0015	0.0723	0.2500	0.2500
B → A	11	0.0042	0.2025	0.0535	0.1591
B → C	9	0.0012	0.1253	0.0358	0.0038
C → B	4	0.0119	0.0810	0.0251	0.0459
C → D	4	0.0057	0.2497	0.3000	0.2286
D → C	3	0.0042	0.0710	0.0000	0.0000

Table 2 of the collaborators' September 2026 worksheet, annualised. Model values are the calibrated or fixed values in force (B→C and C→D apply at all ages).

Stage-D mortality depends on its proxy: the two definitions differ up to 19-fold

		Worksheet stage-D death rate	
	Model input (fixed)	Literature proxy	GBD heart-failure proxy
Indonesia	11.9%	2.4%	11.9%
Uganda	6.0%	0.3%	6.0%

Worksheet rates divide GBD 2023 RHD deaths by an estimated stage-D count; collaborator worksheet, October 2026. All ages, both sexes.

Economic parameters differ by country in GNI, growth and unit costs

Parameter	Indonesia	Uganda
Discount rate	3%	3%
Discount base year	2026	2026
GNI per capita, Atlas method (VSL anchor)	US$4,020	US$810
GNI / VSL anchor year	2019	2019
VSL multiple of GNI per capita	30	30
VSL in the anchor year	US$120,600	US$24,300
Real GDP per capita growth (carries VSL)	3.0%	3.2%
DALYs per RHD death	30	30
Cost per screening encounter	US$1.10	US$0.40
Cost per person-year on BPG	US$110	US$40
Cost per surgery	US$9,000	US$5,000
Cost per confirmatory echo	not costed	not costed

As saved with the economic results. GNI: World Bank WDI NY.GNP.PCAP.CD. DALY: deaths only, no years lived with disability.

Annual incremental cost peaks in 2051 (Indonesia) and 2062 (Uganda); the scale-up ramp ends in 2050

	Indonesia, US$ m				Uganda, US$ m			
Year	Ref.	Scale-up	Incr.	Confirm.	Ref.	Scale-up	Incr.	Confirm.
2030	38.3 m	39.9 m	1.6 m	1,730	6.0 m	6.3 m	0.3 m	1,388
2040	39.1 m	46.3 m	7.2 m	6,066	8.2 m	10.2 m	2.0 m	5,854
2050	37.1 m	51.0 m	13.9 m	9,805	10.2 m	14.9 m	4.7 m	11,518
2060	33.2 m	45.9 m	12.8 m	5,560	11.7 m	17.3 m	5.7 m	7,951
2070	28.5 m	39.0 m	10.5 m	3,711	12.4 m	17.9 m	5.5 m	5,911
2080	23.9 m	32.5 m	8.6 m	2,821	12.4 m	17.2 m	4.9 m	4,784
2090	19.7 m	26.9 m	7.2 m	2,268	11.6 m	15.8 m	4.2 m	3,945
2100	16.1 m	22.1 m	6.0 m	1,831	10.5 m	14.0 m	3.5 m	3,190

Undiscounted annual programme cost by arm (screening + BPG + surgery), selected years; Confirm. = incremental confirmatory echoes, a volume, not priced.

The submitted abstract used earlier runs and a different specification; the preliminary runs do not reproduce it

	Uganda (low-income illustration)		Indonesia (upper-middle-income)	
	Abstract	Saved run	Abstract	Saved run
RHD deaths averted by 2050	7,900	66	23,000	757
Death-rate reduction in 2050	30%	0.6%	24%	0.4%
Benefit–cost ratio	1.28	0.07	1.42	1.16

Abstract values parsed from the submitted WCRHD 2026 abstract (hypothetical countries, earlier runs). Saved-run values are sums to 2050 of the current preliminary outputs.

No sensitivity analysis exists yet; these analyses are planned

- **Structural:** adult progression and normalisation; prior-A in prevalence; direct advanced entry in children
- **Calibration:** prevalence : death weights; death-trend handling; target basis
- **One-way:** SAP hazard ratios (child and adult); BPG initiation, retention, adherence; confirmation completion; screening window target
- **Economic:** confirmation unit cost; VSL multiple; discount rate; unit costs
- **Probabilistic:** transitions and incidence multipliers within bounds

The pipeline is deterministic; none of the results in this deck carry uncertainty intervals.

Six discrepancies between documents, configuration and code are logged

Item	Detail	Action
Indonesia incidence trend	Saved run 0.985; scripts/00_run_all.R 0.985 (consistent)	None
Pipeline control	PIPELINE_RUN_THROUGH = 08: 05–08 run under a user override of the failed fit	Recalibrate
Project guide	CLAUDE.md still describes the October (pre-first-diagnosis) structure	Update
Engine tests	tests/ target an earlier engine signature and output columns	Update tests
State diagram file	docs/ PNG predates the first-diagnosis structure; the deck draws its own	Redraw
Confirmation cost	Volume tracked in 06 and 08; no unit cost in 00 block J	Decide

Provenance (1/2): calibration and health results trace to saved objects

Result	Script	Artifact	Object	Shape	Transform
Fit, statuses	04b	data/<C>/calibrated_rhd_parameters.rds	$stage_calibration$fit_tolerance_summary, $fit_by_year, $statuses	years × quantities	ranges
Parameters, bounds	04b	same	$stage_calibration$parameters, $bounds	39 rows	none
Mechanisms	04b	same	$fit_by_year targets, $fit_by_group_year	age group × year	ratios
Validation	01, 04b, 06	data/<C>/temp_baseline_rates_gbd.rds; same; output/<C>/out_model/<C>.rds	GBD Number; $fit_by_year; $wsd, $stages (ref)	year	sums
Deaths averted	06	output/<C>/out_model/<C>.rds	wsddead	101 ages × 2 sexes × 75 years × 2 arms	ref − sap; cumulative
Care states, volumes	06	same	$stages undiagnosed_end, diagnosed_end, n_dx_*, n_on_bpg_*	same; persons	sums by arm and year

Provenance (2/2): service, cost and economic results trace to saved objects

Result	Script	Artifact	Object	Shape	Transform
Care inputs, seed	05	data/<C>/baseline_state.rds	$states[[1]]$coverage, $seed_state	vectors of 75 years	none
Service volumes	06	output/<C>/out_model/<C>.rds	$stages n_screened, n_confirm_completed, n_on_optimal_treatment	persons per year	sum by arm and year
Cost composition	06, 08	same + rhd_economic_results.rds	$stages volumes × $params unit costs	US$	× discount factor; sap — ref
Economic summary	08	output/<C>/tables/rhd_economic_results.rds	$summary, $incremental	1 row per location	none
Uncosted confirmation	06, 08	same	$budget_impact, $params	per year	sums
Literature, stage-D proxies	none	dev/*.xlsx	Table 2; All-ages rows	probabilities	min, max
Alternatives tested	none	dev/checkpoint_first_diagnosis_2026-11-05.md	Section 10 table	text	parsed

<C> = Indonesia or Uganda; paths relative to the repository root. The deck reads these files and writes nothing.

Rendering needs only the saved artifacts, R and a XeLaTeX toolchain

- Render: `rmarkdown::render("reports/slideck-technical.Rmd")` with XeLaTeX and Calibri; set `RHD_REPO_ROOT` when rendering a copy outside the repository
- Inputs: artifacts of scripts 01–08 for both countries; a missing file stops the render and names its producing script
- Checks: engine and specification, seed = bridged state, reference care = reference arm, care trajectories, SAP trace, nesting, volumes and economics agree for each country
- Software: R version 4.5.1 (2025-06-13 ucrt); rmarkdown 2.29; knitr 1.50; data.table 1.17.8; ggplot2 4.0.3; kableExtra 1.4.0
- Saved: calibration 2026-11-05 / 2026-11-05; model outputs 2026-11-05 / 2026-11-05 (Indonesia / Uganda)

References

Coates, M. M., Sliwa, K., Watkins, D. A., Zühlke, L., Perel, P., Berteletti, F., Eiselé, J. L., Klassen, S. L., Kwan, G. F., Mocumbi, A. O., Prabhakaran, D., Habtemariam, M. K., & Bukhman, G. (2021). An investment case for the prevention and management of rheumatic heart disease in the african union 2021–30: A modelling study. *The Lancet Global Health*, *9*, e957–e966. https://doi.org/10.1016/S2214-109X(21)00199-6

Rwebembera, J., Marangou, J., Mwita, J. C., Mocumbi, A. O., Mota, C., Okello, E., Nascimento, B., Thorup, L., Beaton, A., Kado, J., Kaethner, A., Kumar, R. K., Lawrenson, J., Marijon, E., Mirabel, M., Nunes, M. C. P., Piñeiro, D., Pinto, F., Ralston, K., … Reményi, B. (2023). 2023 world heart federation guidelines for the echocardiographic diagnosis of rheumatic heart disease. *Nature Reviews Cardiology 2023 21:4*, *21*, 250–263. https://doi.org/10.1038/s41569-023-00940-9